

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Our Ref: (T) AJ/mp/ [MED-ID]

[DATE]

Clinic Letter : [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division of Unscheduled Care

Care Group – Medicine & Elderly Care Gastroenterology

SGH, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS], Southampton, [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

I reviewed this lady in clinic today. She continues on her reducing course of Prednisolone and is currently on 25 but was dropping down to 20 mg. Since stopping her Asacol she finds that her pain has returned and she has some streaks of blood in her stools. I have checked the result level which is fine and I have started her on a low dose of [NAME] 75 mg at night and have written separately asking you to prescribe this. It is going to take three or four months for this to reach therapeutic levels in her body and she should continue taking the reducing course of Prednisolone. I asked her to restart the Asacol at 800 mg tds and this helps with her symptoms.

We will see her again in a month's time. I have given her blood forms to have weekly FBCs, Us and amylase done and I will see her again in a month's time. I will be checking the FBCs myself and I am grateful if you could also keep an eye on them as [NAME] can cause pancytopenia. When we see her in a month we will increase her [NAME] to a higher dose as long as she is tolerating it.

Yours sincerely

[PROVIDER-NAME]

Locum Consultant Gastroenterologist